

Lincolnshire Knee Clinic

Consultant-Led Specialist Knee Care

ACL Reconstruction Recovery Guide

A patient guide for ACL reconstruction recovery

Introduction

Reconstruction of the Anterior Cruciate Ligament (ACL) requires structured, phase-based rehabilitation to restore stability, strength, and range of motion. This guide outlines the five distinct recovery phases.

Phase 1: Protect Graft & Restore Extension (Weeks 1 - 2)

- Prioritise full knee extension (straightness). Rest with a towel roll under your ankle, not your knee.
- Actively perform quadriceps sets and straight leg raises in your brace (if prescribed).
- Use crutches at all times to protect the joint and control weight-bearing.

Phase 2: Gradual Flexion & Gait Normalisation (Weeks 3 - 6)

- Aim to bend the knee to 90 degrees by week 2, progressing to 120 degrees by week 6.
- Gradually transition to full weight-bearing and off crutches once you can walk without a limp.
- Begin closed-chain strength exercises like wall slides and mini-squats as guided by physiotherapy.

Phase 3: Strengthening & Balance (Weeks 6 - 12)

- Focus on single-leg strength and neuromuscular control (proprioception).
- Introduce gym-based exercises: leg press, hamstring curls, and stationary cycling with resistance.
- Work on control of the knee during dynamic movements, preventing it from collapsing inwards.

Phase 4: Running & Agility (Months 3 - 6)

- Initiate straight-line jogging once strength testing shows minimal deficit (usually >75% of uninjured leg).
- Begin lateral cutting, pivoting, and jumping drills under supervision.
- Focus on shock absorption and landing mechanics.

Phase 5: Return to Sport (Months 6+)

- Progress to full, unrestricted contact sport and training drills.
- Graduation requires passing a comprehensive functional testing battery (strength, hopping, psychological readiness).

- Continue maintenance strength and conditioning to prevent reinjury.